

D3 — ED Physician Note

Dr. K. Washington, Emergency Medicine | Emergency | 02/17/2026 18:32

Patient: Pemberton, Harold J.**MRN:** 847-29-3156**DOB:** 04/12/1951**Allergies:** Sulfa drugs (rash)

EMERGENCY DEPARTMENT PHYSICIAN NOTE

Patient: Harold J. Pemberton | MRN: 847-29-3156
Date: 02/17/2026 18:32 | Provider: Dr. K. Washington, Emergency Medicine

CHIEF COMPLAINT: Shortness of breath x 3 days, productive cough

HISTORY OF PRESENT ILLNESS:
74 y/o male with PMH significant for COPD, CHF (EF 30%), afib, HTN, T2DM, CKD 3b presents with 3 days of worsening dyspnea, productive cough with yellow-green sputum, and wheezing. Reports low-grade fever at home (100.4°F). Denies chest pain. Reports some mild bilateral ankle swelling, which he states is "usual for me."

ALLERGIES: Sulfa drugs – RASH

MEDICATIONS (per patient report – patient unable to recall complete list):

- "Blood thinner" (thinks apixaban)
- "Heart pills" – unclear
- "Sugar pill" – likely metformin
- "Breathing machine" – home inhalers
- "Cholesterol pill"
- "Thyroid pill"
- Several others he cannot recall

VITAL SIGNS:
HR 98 | BP 148/82 | RR 24 | Temp 100.4°F | SpO2 89% on RA → 94% on 2L NC | Wt 82 kg

PHYSICAL EXAMINATION:
General: Elderly male in mild respiratory distress, speaking in short sentences
Lungs: Diffuse bilateral expiratory wheezing, decreased breath sounds at bases, no rales appreciated
CV: Irregularly irregular, no murmurs appreciated in ED
Ext: Bilateral LE edema 1+ (patient states baseline)
Neuro: Alert and oriented x 4

DIAGNOSTICS:
- CXR: Hyperinflated lung fields. No acute consolidation. Bilateral small pleural effusions (likely chronic).

- ECG: Atrial fibrillation with controlled ventricular rate, no acute ST changes
- Labs: WBC 13.2, Hgb 11.2, Plt 198, Na 136, K 4.2, Cl 101, CO2 28, BUN 32, Cr 1.6 (baseline per records ~1.5), Glucose 186, BNP 480, Lactate 1.1, Troponin <0.01
- Procalcitonin: 0.18 (low)

MEDICAL DECISION MAKING:

Presentation most consistent with acute COPD exacerbation with superimposed acute bronchitis. BNP 480 – in the context of known CHF with EF 30%, this is likely near-baseline and does not suggest acute decompensation at this time. Low-grade fever and productive cough suggest infectious trigger. CXR without consolidation argues against pneumonia.

ED MANAGEMENT:

- Nebulized albuterol 2.5 mg + ipratropium 0.5 mg x 2
- Methylprednisolone 125 mg IV x 1
- 1L Normal Saline bolus (patient appears mildly dry, poor PO intake x 3 days)
- Placed on 2L NC with improvement to SpO2 94%

DISPOSITION: Admit to medicine for COPD exacerbation management.

NOTE: Patient's family member (daughter) arrived after admission. She provided a handwritten list of patient's home medications to the floor nurse. This list has been scanned into the chart but was not reviewed by the ED team prior to disposition.

Electronically signed by: Dr. K. Washington, MD

Date: 02/17/2026 20:15